

DISCHARGE SUMMARY

Discharge Date: 2026

[PATIENT]

MRN	[MRN]	DOB	[DOB]	Age / Sex	74 M / Male
Admitted	2026	Discharged	2026	Attending	[PROVIDER], MD — Cardiology

DIAGNOSES

Admitting Diagnosis:
Acute decompensated heart failure with volume overload

- Discharge Diagnosis / Diagnoses:
- Acute decompensated heart failure (HFrEF, EF 28%)
 - Hypertensive urgency, now controlled
 - Chronic kidney disease stage 3a (GFR 48 at discharge)
 - Type 2 diabetes mellitus, stable

HISTORY OF PRESENT ILLNESS

[PATIENT] is a 74-year-old male with known ischemic cardiomyopathy (EF 28%) who presented to the ED with a 5-day history of progressive dyspnea on exertion, bilateral lower extremity edema to the mid-shin, orthopnea (3-pillow), and a 6 lb weight gain. He reported dietary indiscretion over the Memorial Day weekend and admitted to missing two doses of his furosemide. BNP on presentation was 2,840 pg/mL. Chest X-ray showed pulmonary vascular congestion with small bilateral pleural effusions. He was admitted to the Heart Failure Service for IV diuresis and optimization.

HOSPITAL COURSE

Diuresis: IV furosemide 80 mg BID initiated on admission. Net negative fluid balance of 4.2 L over the first 48 hours. Patient transitioned to oral torsemide 40 mg daily on hospital day 3. Repeat BNP at discharge: 680 pg/mL. Weight at discharge: 182 lbs (6 lbs below admission weight).

Cardiac: Telemetry monitoring throughout. No sustained arrhythmia. Echocardiogram confirmed EF 28%, moderate mitral regurgitation, dilated LV. Sacubitril/valsartan uptitrated from 24/26 mg to 49/51 mg BID given improved renal function.

Renal: Creatinine peaked at 1.6 mg/dL on day 2 (baseline 1.3). Improved to 1.4 at discharge. Metformin held during admission and at discharge pending renal recheck.

Hypertension: Blood pressure controlled with amlodipine 10 mg and uptitrated carvedilol 12.5 mg BID. Systolic BP 118–130 range at discharge.

PAST MEDICAL HISTORY

- Ischemic cardiomyopathy — diagnosed 2019

- Type 2 diabetes mellitus — on metformin
- Hypertension — 15 years
- CKD stage 3a
- Prior MI (2018) — stented LAD

DISCHARGE MEDICATIONS

MEDICATION / DOSE	ROUTE / FREQ	NOTES
Torsemide 40 mg	PO daily	NEW — replaces home furosemide 40 mg. Weigh daily.
Sacubitril/valsartan 49/51 mg	PO BID	UPTITRATED from 24/26 mg. Monitor BP.
Carvedilol 12.5 mg	PO BID	UPTITRATED. Hold if HR < 50 or SBP < 90.
Amlodipine 10 mg	PO daily	Continue. Monitor for edema.
Spironolactone 25 mg	PO daily	Continue. Check BMP at follow-up.
Atorvastatin 40 mg	PO nightly	Continue.
Aspirin 81 mg	PO daily	Continue.
Metformin 1000 mg	PO BID	HELD — resume only after renal recheck confirms GFR stable.

■ PENDING ITEMS — PCP ACTION REQUIRED

- Repeat BMP in 5–7 days to assess creatinine and potassium response to torsemide.
- Metformin HELD at discharge — must be restarted only after renal function confirmed stable.
- Lipid panel overdue — please check at PCP follow-up.

FOLLOW-UP PLAN

SPECIALTY	FOLLOW-UP DATE	INSTRUCTIONS
Cardiology / HF Clinic	2026	Assess volume status, weight trend, BNP. Consider further up titration of sacubitril/valsartan.
Primary Care	2026	BMP recheck (creatinine, potassium). Resume metformin only if GFR >=45.
Device Clinic / ICD	2026	Routine ICD interrogation — battery at 3.4 years remaining.
Dietitian	2026	Reinforcement of 2g sodium / 1.5L fluid restriction diet counseling.

DISPOSITION & INSTRUCTIONS

Discharged home with spouse. Patient and wife educated on daily weights, sodium restriction, fluid restriction, and escalation plan: call cardiology if weight increases >3 lbs in 24 hours or >5 lbs in 1 week, or if dyspnea worsens. Written action plan provided. Patient verbalized understanding. Functional status at discharge: ambulating independently, no supplemental O2 required. Discharged in stable condition.

Electronically signed by: [PROVIDER], MD, FACC — Cardiology, [FACILITY]

Date/Time: 2026 | [FACILITY]

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